

D10 – Office Visit Note

P.A. Kim, MD | Family Medicine | 09 Apr 2013

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 04/09/2013 | Provider: P.A. Kim, MD

Visit Type: Established Patient / Follow-Up

Practice: Oak Harbor Family Medicine (TRICARE), Oak Harbor WA

CHIEF COMPLAINT: Recurrent headaches. BPH workup follow-up. Chronic condition management.

HISTORY OF PRESENT ILLNESS:

57-year-old active duty USN with new and worsening headache pattern over the past several months. Bifrontal pulsating quality, moderate to severe in intensity. Associated photophobia, phonophobia, and nausea during episodes. Episodes occur approximately every 6 to 8 weeks and last 6 to 18 hours. Partially abortive with NSAIDs but symptom burden is significant during attacks - patient reports needing to lie down in a dark room. No clear trigger. No focal neurological symptoms between episodes. No worst-headache-of-life. No fevers, neck stiffness, or neurological deficits.

Also seen by urology in interim for BPH workup. Returns for combined follow-up.

CURRENT MEDICATIONS:

- Omeprazole 40 mg PO daily
- Metoprolol succinate 25 mg PO daily
- Rivaroxaban 20 mg PO daily
- Naproxen 500 mg PO BID PRN
- Flunisolide nasal spray PRN
- CPAP 10 cmH2O nightly

PHYSICAL EXAMINATION:

Vitals: HR 70 | BP 132/80 | Wt 88 kg

General: NAD today, no active headache.

HEENT: Atraumatic. No scalp tenderness. No temporal artery tenderness. No papilledema on fundoscopy.

CV: Rate-controlled, regular.

Neuro: Cranial nerves II-XII intact. Strength 5/5 bilaterally. No drift. Sensation intact. Coordination normal. Gait normal.

ASSESSMENT & PLAN:

1. Recurrent headaches - bifrontal pulsating, moderate to severe, associated with photophobia, phonophobia, and nausea. Episodes approximately every 6 to 8 weeks lasting 6 to 18 hours. Pattern consistent with migraine without aura. No red flags on history or exam. Neurology referral placed for confirmation. Start topiramate 25 mg PO daily for prophylaxis. Counseled re: potential cognitive side effects and renal stone risk.
2. BPH - urology follow-up (Dr. Tran, 14 Jan 2013): PSA 4.8 ng/mL (elevated). 12-core TRUS biopsy recommended - pending scheduling. Start tamsulosin 0.4 mg PO daily for symptomatic management. Counseled re: orthostatic side effects.
3. Erectile dysfunction - urology continuing workup. No change per

patient.

4. Obstructive Sleep Apnea - CPAP compliance good per device download. Symptoms improved.
5. Atrial fibrillation - rate-controlled on metoprolol. Cardiology stable. Rivaroxaban continued.
6. GERD - symptoms persisting on PPI. GI re-referral placed to Dr. Abara.

Follow-up: Neurology pending. TRUS biopsy result pending. PCP RTC 3 months.

Electronically signed by: P.A. Kim, MD
Date: 04/09/2013 13:55

★ SYNTHETIC TRAINING DOCUMENT – AI RESEARCH USE ONLY – NOT AN OFFICIAL MEDICAL RECORD ★